

AZOSIN® XL 10mg PROLONGED RELEASE TABLET

Alfuzosin Hydrochloride 10mg

Presentation:

AZOSIN XL 10mg Prolonged Release Tablet is a prolonged-release oral tablet, presented as light yellow round tablet, engraved "STD" on one side "177" on the other side.

Composition:

Each prolonged release tablet contains Alfuzosin Hydrochloride 10mg.

Pharmacodynamic:

Alfuzosin is an orally active derivative of quinazoline. It is a selective antagonist of post-synaptic alpha-1-adrenoreceptors. The selectivity of alfuzosin for the alpha-1-adrenoreceptors is located in the prostate, the bladder trigone and the urethra.

Alpha-blockers, through a direct action on the smooth muscle of the prostate tissue, reduce the subvesical obstruction.

Studies have shown that alfuzosin reduces urethral pressure thereby lowering resistance to urine flow during micturition. Study also showed a greater effect on urethral pressure than on the effect on blood pressure. In patients suffering from benign prostatic hypertrophy, alfuzosin was shown:

- significantly increased urine flow by an average 30% in patients with a flow rate ≤ 15 mL/sec. This improvement was observed from the first dose.
- significantly decreased detrusor pressure and increased the volume producing the sensation of the need to urinate.
- significantly reduced residual urine volume.

These effects result in an improvement in the irritative and obstructive urinary symptoms. They do not have detrimental effect on sexual function.

Pharmacokinetics:

Alfuzosin hydrochloride is approximately 90% bound to plasma proteins. Alfuzosin is extensively metabolized by the liver with excretion in the urine of only 11% of the unchanged compound. The majority of metabolites (which are inactive) are excreted in the faeces (75-90%). The pharmacokinetic profile of alfuzosin is not modified in the case of chronic cardiac insufficiency.

Prolonged-Release Formulation: The mean value of relative bioavailability is 104.4% after administration of a dose of 10 mg, compared with that of the immediate-release formulation at a dosage of 7.5 mg (2.5 mg 3 times daily) in middle-aged healthy volunteers. The peak plasma concentration is reached 9 hours after administration as against 1 hour for the immediate formulation.

The apparent elimination half-life is 9.1 hours.

Studies have shown that the bioavailability is increased when the product is administered after a meal (see Dosage & Administration).

The pharmacokinetic properties (C_{max} and AUC) are not increased in elderly patients, compared with middle-aged healthy volunteers.

The mean values of C_{max} and AUC are moderately increased in patients with moderate impairment of renal function (creatinine clearance ≥ 30 mL/min) without effect on the elimination half-life compared with patients with normal renal function.

Dosage adjustment is not necessary in patients with impaired renal function with creatinine clearance > 30 mL/min.

Indications:

- Treatment of functional symptoms of benign prostatic hypertrophy.
- Adjuvant treatment to a catheter in acute urinary retention related to benign prostatic hypertrophy.

Dosage and Administration:

Oral use.

The recommended dosage is 1 tablet daily to be taken immediately after the evening meal. The tablet must be swallowed whole with a glass of water.

Adjuvant treatment to a catheter in acute urinary retention related to benign prostatic hypertrophy:

The recommended dosage is one 10 mg tablet per day, to be taken after a meal, from the first day of catheterization onwards. The treatment is administered for 3 to 4 days, i.e. 2 to 3 days while the catheter is in place and 1 day after it is removed.

Contraindications:

This medicinal product must not be administered in the following situations:

- hypersensitivity to alfuzosin and/or any of the other ingredients;
- postural hypotension;
- liver failure;
- severe kidney failure (creatinine clearance < 30 mL/min);
- in combination with ritonavir.

Special Warnings and Precautions:

As with all alpha-1-blockers in some subjects, in particular patients receiving antihypertensive medications, postural hypotension may occur during the first few hours following administration of the medicinal

product possibly accompanied by symptoms (dizziness, fatigue, sweating). In such cases, the patient should lie down until the symptoms have completely disappeared.

These effects are transient and do not usually prevent the continuation of treatment after adjustment of the dose. The patient should be warned of the possible occurrence of such events.

In patients with coronary insufficiency specific anti-anginal therapy should be continued, but if the angina reappears or worsens AZOSIN XL should be discontinued.

Experience in patients with severe renal impairment is limited and cautious use in these patients is recommended.

Patients should be warned that the tablet should be swallowed whole. Any other mode of administration, such as crunching, crushing, chewing, grinding or pounding to powder should be prohibited. These actions may lead to inappropriate release and absorption of the drug and therefore possible early adverse reactions.

The excipient hydrogenated castor oil may cause stomach upset and diarrhoea.

The 'Intraoperative Floppy Iris Syndrome' (IFIS, a variant of small pupil syndrome) has been observed during cataract surgery in some patients on or previously treated with tamsulosin. Isolated reports have also been received with other alpha-1 blockers and the possibility of a class effect cannot be excluded. As IFIS may lead to increased procedural complications during the cataract operation current or past use of alpha-1 blockers should be made known to the ophthalmic surgeon before the eye surgery.

Effect on ability to drive and use machine:

Particular caution must be exercised by drivers and machine operators because of the risk of orthostatic hypotension, particularly at the beginning of treatment with alfuzosin.

Use in pregnancy & lactation:

The therapeutic indication is not intended for women.

The safety of alfuzosin during pregnancy and the passage in the breast milk are not known.

Drug Interactions:

Inadvisable Combinations:

- Antihypertensive alpha-receptor blockers (prazosin, trimazosin, urapidil, minoxidil):
Enhanced hypotensive effect. Risk of severe postural hypotension.
- Ketoconazole, itraconazole, ritonavir, clarithromycin, erythromycin:
Risk of increased plasma alfuzosin concentrations and increased undesirable effects.

Combination Requiring Precautions for Use:

- Phosphodiesterase type 5 inhibitors (sildenafil, tadalafil, vardenafil):
Risk of postural hypotension, particularly in elderly subjects. Treatment should be initiated at the lowest recommended dose and adjusted gradually if necessary.

Combinations to be Taken Into Consideration:

- Antihypertensives except alpha-receptor blockers:
Enhanced hypotensive effect. Higher risk of postural hypotension.
- Nitrates, nitrites and related drugs (isosorbide dinitrate, isosorbide, linsidomine, molsidomine, nicorandil, nitroglycerin):
Increased risk of hypotension, particularly postural hypotension.

Adverse Reactions:

The undesirable effects observed most commonly in patients treated with alfuzosin have been the following:

Gastrointestinal disorders (nausea, gastric pain, diarrhoea), vertigo, dizzy sensations, malaise, and headache.

More rarely, the following have been reported:

Orthostatic hypotension, syncope, tachycardia, palpitations, chest pain (see Precautions), asthenia, drowsiness, oedema, flushing, dry mouth, skin rash, and pruritus.

Overdosage:

In case of overdosage, the patient should be hospitalized, kept in the supine position, and conventional treatment of hypotension should take place. Alfuzosin is not dialysable because of its high degree of protein binding.

Storage Conditions:

Store below 30°C.

Packing Available: 30 tablets/ box.

Date of Revision: June 2011.

Manufacturer:

Standard Chem. & Pharm. Co. Ltd., 2nd Plant,
No. 154, Kaiyuan Rd., Sinying Disirict,
Tainan City, 73055, Taiwan.